

Admin allocation: script requests, documents in Docman, tasks, medical reports

The practice staff allocate admin work as follows:

Baseline admin rules

- Admin only goes to regular doctors, never locums
- no admin if Drs are on leave for 1 week or more (this includes sick leave and maternity leave)
- To help keep continuity, routine admin goes to the usual GP where possible (see below for details)
- If admin is being shared between all doctors, it is done based on the next session, not the current session.

Explanation: doctors work 2 sessions a day (am and pm). 1pm is the cut off between am and pm. Morning work should be allocated between the afternoon doctors so that at 1pm they can see how much admin workload they have and plan how to fit it into their admin time. This avoids sending doctors work after they have done their admin as the doctors should not have to go back to look at their tasks/docman/scripts etc. Work allocated in an afternoon is shared between the doctors working the next morning. Only the on call or admin doctors should get urgent 'same session' work.

Tasks & Docman – these are routine work. If urgent (same day) it needs to be added to the on-call Drs session – if the pt needs an appt (inc phone) it must be added as an appt-if unsure speak to the on call doctor – same session as query arose. To allocate tasks or docman please use the following priorities:

- 1 – Send to Dr dealing with problem (eg: results of a test or query from a consultation etc)
- 2 – Send to Usual GP
- 3 – share between Drs in work (next session, 1pm cut off), including remote/special/training sessions

Scripts - Baseline script rule – only goes to Drs working next session. To allocate scripts please use the following priorities:

- 1 – pick usual GP if available
- 2 – split between GPs in work, including remote/special/training sessions

Lab results – These are reviewed by the Business Admin team using detailed protocols. Some will be forwarded to doctors using the following priority:

- Requesting clinician – this includes Trainees and UCT members
- Usual GP
- Split between GPs in work (next session)

Medication reviews – These are allocated to the Usual GP (if on any type of leave for over 2 weeks ask a manager for alternative allocation process). Staff will send a blank task to the Usual GP using the title 'Prescription' which indicates that a medication request has been actioned by the team but that the medication review date is overdue. The GP should review all medication and check that any associated bloods/checks have been done. Review dates can be set for any period of time but if there are no concerns that necessitate an earlier review, dates are usually set for 2 months after the patient's birthday by which time chronic disease annual reviews should have been completed. Please see the Medication Review Protocol (on Practice Index) for full details.

Private work (reports/forms/letters) – These are allocated to the Usual GP. If Salaried GPs complete any private work in their own time, they should indicate this on the front page that is included with the form. Payment will then be made to individuals via the payroll.

Doctors – if you feel that a colleague is better placed to action a form/admin request you need to liaise with that doctor via patient task. This enables admin staff to keep track of who is responsible for admin. Please do not pass it back to admin staff to be reallocated.